

Dynamic Guideline: WHO-Based Low Back Pain - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

Primary WHO Guideline for This Report

- **Guideline Title:** *WHO guideline for non-surgical management of chronic primary low back pain in adults in primary and community care settings*
- **Publication Date:** **7 December 2023**
- **Status as of 29 April 2026:** Most current WHO clinical guideline identified for low back pain; no newer WHO replacement guideline was identified in the supplied evidence.
- **Target Population:** Community-dwelling adults with **chronic primary low back pain**, including older adults, with or without spine-related leg pain.
- **Definition:** Chronic primary low back pain is persistent or recurrent low back pain lasting **more than three months** that is **not reliably attributed to an underlying disease process or structural lesion** [WHO guideline PDF](#).
- **Setting:** Primary and community care.
- **Scope:** Non-surgical interventions deliverable in primary/community care.
- **Excluded:** Surgery, invasive procedures usually delivered in secondary/tertiary care, and workplace interventions [WHO publication page](#).

WHO describes this as its **first-ever guideline** for managing chronic low back pain in primary and community care settings [WHO news release](#).

Burden of Disease

WHO identifies low back pain as the **leading cause of disability globally**. In 2020, approximately **619 million people**, or about **1 in 13 people**, experienced low back pain, a **60% increase from 1990**. Cases are projected to rise to **843 million by 2050**, with the largest increases expected in Africa and Asia [WHO news release](#).

Exactly 5 WHO Documents/Guidelines Synthesized

No.	WHO Document	Date / Status	Relevance to Low Back Pain	Role in This Report
1	WHO guideline for non-surgical management of chronic primary low back pain in adults in primary and community care settings	7 December 2023	Direct, disease-specific clinical guideline	Primary guideline source WHO publication page
2	WHO Low back pain fact sheet	19 June 2023	Disease definition, symptoms, duration	Diagnostic/contextual support WHO low back pain fact sheet

No.	WHO Document	Date / Status	Relevance to Low Back Pain	Role in This Report
			categories, public-health burden	
3	WHO Package of Interventions for Rehabilitation – Low back pain module	Current WHO rehabilitation package	Rehabilitation interventions and resources for adults with chronic primary low back pain	Implementation and rehabilitation support WHO rehabilitation package PDF
4	WHO musculoskeletal health / rehabilitation materials	Updated WHO fact-sheet context	Low back pain as major musculoskeletal contributor and rehabilitation priority	Health-system and burden context WHO musculoskeletal health fact sheet , WHO rehabilitation fact sheet
5	Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline (2022)	WHO evidence brief promoted via 11 December 2025 launch	Not a low back pain clinical guideline; relevant only to WHO guideline-development methodology, inclusivity, and stakeholder engagement	Process-methodology reference; the latest WHO document matching the specified title WHO publication page , WHO webinar page

Important scope clarification: The document titled *Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline (2022)* is **not a low back pain guideline**. It is included here only because the prompt specifically required identification of the most recent WHO item with that title; it informs stakeholder-engagement methodology rather than low back pain diagnosis or treatment.

1.2 Diagnostic and Clinical Assessment Principles

Although the WHO 2023 guideline focuses on management rather than diagnostic workup, current evidence-based care requires initial clinical triage.

Duration Categories

WHO classifies low back pain by duration as follows:

Category	Duration
Acute low back pain	<6 weeks
Subacute low back pain	6–12 weeks
Chronic low back pain	>12 weeks

Source: [WHO low back pain fact sheet](#)

Initial Assessment

Adults with chronic low back pain should receive:

- History and physical examination.
- Assessment for **red flags** suggesting serious disease.
- Assessment for **yellow flags**, including psychological, environmental, occupational, and social factors associated with chronicity and disability.
- Neurologic examination when radicular symptoms are present.
- Avoidance of routine imaging unless red flags, neuromuscular deficit, or failure of conservative therapy are present [AAFP](#).

1.3 Key WHO Recommendations

WHO considered **37 non-surgical interventions** across five classes:

1. Structured and standardized education
2. Physical interventions
3. Psychological interventions
4. Medicines
5. Multicomponent interventions

The guideline produced **24 recommendations, one good practice statement**, and **12 no-recommendation decisions** because of inadequate, absent, or equivocal evidence [WHO guideline PDF](#), [PMC commentary](#).

GRADE Table: WHO-Based Clinical Recommendations

Recommendation	Direction	GRADE Evidence Quality	Strength
Provide holistic, person-centred, integrated, coordinated, equitable, non-stigmatizing care addressing physical, psychological, and social contributors.	For	Moderate	Strong
Offer structured and standardized education and/or advice as part of care.	For	Low	Conditional
Encourage activity and avoid bed rest as part of non-surgical care.	For	Moderate	Strong
Offer structured exercise therapies or programmes tailored to the person.	For	Low	Conditional
Consider cognitive behavioural therapy as part of care.	For	Low in WHO evidence base; upgraded post-guideline to Moderate by newer evidence	Conditional
Consider operant therapy as part of psychological care.	For	Low / Very Low	Conditional
Consider mindfulness-based interventions as part of biopsychosocial care where available and acceptable.	For	Low in WHO evidence base; upgraded post-guideline to Moderate by newer evidence	Conditional

Recommendation	Direction	GRADE Evidence Quality	Strength
Consider needling therapies where trained providers and safe systems are available.	For	Low	Conditional
Consider spinal manipulative therapy as part of a broader care plan, not as stand-alone mandatory treatment.	For	Low	Conditional
Consider massage as an adjunctive physical intervention.	For	Low	Conditional
Offer non-steroidal anti-inflammatory drugs when medication is needed and risks are acceptable.	For	Moderate	Conditional
Consider topical cayenne pepper / Capsicum frutescens as an adjunct.	For	Low	Conditional
Offer multicomponent biopsychosocial care addressing at least two domains: physical, psychological, and/or social.	For	Low	Conditional
Provide assistive mobility products when needed to support safe mobility and function.	Good practice	Not GRADE-rated	Good Practice Statement
Do not routinely offer traction .	Against	Low / Very Low	Conditional
Do not routinely offer therapeutic ultrasound .	Against	Low / Very Low	Conditional
Do not routinely offer transcutaneous electrical nerve stimulation , or TENS.	Against	Low / Very Low	Conditional
Do not routinely offer lumbar braces, belts, or supports .	Against	Low / Very Low	Conditional
Do not routinely use opioid analgesics for chronic primary low back pain.	Against	Moderate	Conditional
Do not routinely use SNRI antidepressants solely for chronic primary low back pain.	Against	Low / Very Low	Conditional
Do not routinely use tricyclic antidepressants solely for chronic primary low back pain.	Against	Low / Very Low	Conditional
Do not routinely use anticonvulsants for chronic primary low back pain.	Against	Low / Very Low	Conditional
Do not routinely use skeletal muscle relaxants for chronic primary low back pain.	Against	Low / Very Low	Conditional
Do not routinely use systemic glucocorticoids for chronic primary low back pain.	Against	Low / Very Low	Conditional
Do not routinely use Devil's claw / Harpagophytum procumbens .	Against	Low / Very Low	Conditional

Sources: WHO recommendation summaries and materials [WHO guideline PDF](#), [WHO summary brochure](#), [WHO launch slides](#), [MedCentral](#).

1.4 Guideline Development Methodology

Methodological Element	WHO Approach
Guideline-development standard	Developed according to WHO standards for guideline development PMC commentary
Evidence base	Systematic reviews of randomized controlled trials published to mid-2022 PMC commentary
Trial base	367 randomized controlled trials; 193 from 26 high-income countries, 72 from 5 upper-middle-income countries, 72 from 6 lower-middle-income countries, and 30 multinational trials PMC commentary
Intervention scope	37 interventions across education, physical, psychological, medicine, and multicomponent classes WHO guideline PDF
Recommendation output	24 recommendations, one good practice statement, and 12 interventions with no recommendation WHO guideline PDF
Certainty assessment	WHO used certainty-of-evidence assessment consistent with GRADE reporting by recommendation WHO guideline PDF
Values and equity	WHO emphasized person-centred, equitable, integrated care and noted higher adverse-event risk in older adults WHO news release
Stakeholder engagement	WHO guideline-development processes increasingly emphasize lived experience and inclusive participation; the WHO youth-engagement case study illustrates how lived experience can enrich guideline development, although it is not low-back-pain-specific WHO publication page

1.5 Practical WHO-Based Care Pathway

Step 1: Confirm Chronic Primary Low Back Pain

- Pain between the lower ribs and gluteal folds, with or without spine-related leg pain.
- Duration >3 months.
- No reliable attribution to specific structural lesion, inflammatory disease, infection, fracture, malignancy, or other secondary cause.

Step 2: Screen for Serious or Specific Causes

Use clinical judgement and assess for:

- Cancer history or unexplained weight loss.
- Fever, infection risk, immunosuppression.
- Significant trauma or osteoporosis risk.
- Progressive neurologic deficit.

- Cauda equina features.
- Inflammatory back pain pattern.
- Severe or worsening radicular deficit.

Routine imaging is not recommended without red flags, neurologic deficit, or non-response to appropriate conservative care [AAFP](#).

Step 3: Provide Core Non-Pharmacologic Care

- Education and reassurance.
- Encourage movement and self-management.
- Exercise programme adapted to preferences, function, comorbidities, and access.
- Address sleep, mood, distress, fear avoidance, and social barriers.
- Consider CBT, mindfulness, operant therapy, or multicomponent biopsychosocial care.

Step 4: Use Medicines Selectively

- NSAIDs may be used when benefits outweigh gastrointestinal, renal, cardiovascular, and medication-interaction risks.
- Avoid routine opioids and avoid routine use of antidepressants, anticonvulsants, skeletal muscle relaxants, and glucocorticoids solely for chronic primary low back pain.

Step 5: Reassess Function and Participation

Outcomes should include:

- Pain intensity.
- Disability/function.
- Mobility.
- Work, school, family, and community participation.
- Sleep and mood.
- Medication burden, especially opioid exposure.
- Adverse effects.

II. Evidence Summary After WHO Latest Guideline Release (WHO guideline for non-surgical management of chronic primary low back pain in adults in primary and community) (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

Search Strategy

Evidence was synthesized from the supplied 2024–2026 summaries using the following evidence hierarchy:

1. WHO official documents and updates.
2. Systematic reviews and meta-analyses.
3. Randomized controlled trials.
4. Clinical trial registry records.
5. Authoritative clinical summaries and guidelines.

Databases and Sources Represented

- WHO official pages and WHO-hosted PDFs.
- PubMed-indexed literature.
- *The Lancet Rheumatology* systematic review/meta-analysis.
- *BMJ* evidence review.
- *Nature Medicine* randomized trial.
- ClinicalTrials.gov records.
- AAFP clinical review.
- NICE NG59.
- Peer-reviewed implementation commentary.

Inclusion Criteria

Included:

- Adults with chronic primary, chronic non-specific, or chronic low back pain.
- Non-surgical interventions.
- Primary/community-care relevance.
- Evidence published or posted after the WHO guideline release, through **29 April 2026**.
- Systematic reviews, meta-analyses, randomized trials, protocols, trial registries, and implementation analyses.

Excluded:

- Surgical trials.
- Secondary/tertiary invasive procedures unless included only as registry context.
- Acute/subacute low back pain studies unless relevant to prevention of chronicity.
- Non-human or mechanistic-only evidence.

2.2 Key New Studies

GRADE Table: Post-WHO Evidence Update

Study / Evidence Source	Design	Population	Key Findings	GRADE Quality	Clinical Impact
2025 <i>Lancet Rheumatology</i> systematic review/meta-analysis of long-term non-surgical interventions	Systematic review and meta-analysis; 75 trials; 15,395 participants	Non-specific chronic low back pain	CBT reduced long-term pain: MD –7.2, 95% CI –9.8 to –4.6; disability: MD –5.7, 95% CI –7.7 to –3.7. Mindfulness reduced pain: MD –10.0, 95% CI –14.4 to –5.6; disability: MD –9.3, 95% CI –14.4 to –4.1. However, 68% of	Moderate for CBT and mindfulness; Low for several other interventions	Strengthens WHO support for psychological and mind-body interventions PubMed

Study / Evidence Source	Design	Population	Key Findings	GRADE Quality	Clinical Impact
			included studies were high risk of bias.		
2025 <i>BMJ</i> review of non-surgical low back pain care	Evidence review	Low back pain, including chronic low back pain	For chronic low back pain, advice/education, exercise, integrated physical-cognitive therapies, and psychological therapies had favorable benefit-harm balance; benefits likely moderate for exercise and integrated physical-cognitive therapies, small for advice/education and psychological therapies.	Moderate for exercise/integrated physical-cognitive care; Low-Moderate for advice/education and psychological therapy	Supports WHO non-drug-first, biopsychosocial approach BMJ
2026 randomized trial report of mindfulness and CBT in opioid-treated chronic low back pain	Large randomized clinical trial report; 770 adults	Chronic low back pain treated with daily opioids ≥ 3 months	Eight weeks of mindfulness or CBT improved function and quality of life, reduced pain, and reduced opioid dose, with benefits up to 12 months.	Moderate, downgraded because detailed peer-reviewed effect estimates were not extractable from supplied summary	Reinforces avoiding opioid escalation and using CBT/mindfulness in opioid-treated patients Penn State Health News
2025 <i>Nature Medicine</i> Phase 3 VER-01 cannabis extract trial	Randomized placebo-controlled Phase 3 trial	Chronic low back pain	$\geq 30\%$ pain reduction: 54.1% with VER-01 vs 39.5% with placebo; NNTB 6.8; $P < 0.001$. $\geq 50\%$ pain	Moderate for short-term analgesic efficacy; Low for routine implementation because long-term safety,	Promising but insufficient to change WHO recommendations toward routine cannabinoid use Nature Medicine

Study / Evidence Source	Design	Population	Key Findings	GRADE Quality	Clinical Impact
			reduction: 32.2% vs 22.8%; P=0.010. Lower rescue ibuprofen use: 10.5 vs 18.3 tablets; P<0.001.	generalizability, regulation, and comparative effectiveness remain uncertain	
2024 CMAJ systematic review/meta-analysis on clinical course	Systematic review/meta-analysis	Acute, subacute, persistent low back pain	Provides updated prognosis context for low back pain trajectories; relevant to counseling and expectation setting.	Moderate	Supports early education and avoidance of overmedicalization PMC reference list
2025 implementation commentary on WHO guideline	Peer-reviewed implementation analysis	Global health systems	Implementation requires changes at point of care, service models, organizations, and health-system building blocks; linked to universal health coverage, healthy ageing, and Rehabilitation 2030.	Low for clinical efficacy; Moderate for implementation relevance	Supports system-level adoption planning PubMed
OPTIMIZE / NCT03859713	Completed SMART randomized trial; results posted but numerical results not available in supplied snippet	Chronic low back pain	Compared first-line PT and CBT with re-randomization of non-responders; response defined by 50% ODI improvement.	Very Low from supplied data because results not extractable	Await full results before changing recommendations ClinicalTrials.gov NCT03859713
METRIC / NCT05986370	Ongoing randomized trial	Chronic primary low back pain	Lumbar spinal manipulation vs sham vs full-spine spinal manipulation;	No GRADE rating for efficacy; no results	May inform future manipulation recommendations ClinicalTrials.gov NCT05986370

Study / Evidence Source	Design	Population	Key Findings	GRADE Quality	Clinical Impact
			estimated completion 31 December 2026.		
Pain Reprocessing Therapy / NCT05820204	Ongoing/near-complete randomized trial	Chronic back pain	Pain reprocessing therapy vs CBT for chronic pain vs usual care; no results posted in supplied record.	No GRADE rating for efficacy; no results	Future evidence gap ClinicalTrials.gov NCT05820204
Telerehabilitation network meta-analysis protocol	Protocol	Chronic low back pain	Planned systematic review/network meta-analysis; no treatment-effect results.	No GRADE rating for efficacy; protocol only	Evidence pending PMC protocol

2.3 Intervention-Specific Evidence Update

Cognitive Behavioural Therapy

The strongest post-guideline update is for CBT. A 2025 systematic review/meta-analysis found **moderate-certainty evidence** that CBT probably reduces long-term pain and disability in chronic non-specific low back pain:

Outcome	Effect Estimate	GRADE Quality
Pain intensity	MD −7.2; 95% CI −9.8 to −4.6	Moderate
Disability	MD −5.7; 95% CI −7.7 to −3.7	Moderate

Source: [PubMed](#)

Interpretation: This strengthens the WHO conditional recommendation for CBT as part of biopsychosocial care.

Mindfulness-Based Interventions

The same 2025 meta-analysis found **moderate-certainty evidence** for mindfulness:

Outcome	Effect Estimate	GRADE Quality
Pain intensity	MD −10.0; 95% CI −14.4 to −5.6	Moderate
Disability	MD −9.3; 95% CI −14.4 to −4.1	Moderate

Source: [PubMed](#)

A 2026 randomized trial report also found that mindfulness and CBT reduced pain, improved function, and reduced opioid dose among adults with chronic low back pain receiving daily opioids [Penn State Health News](#).

Interpretation: Mindfulness should be considered a reasonable option when accessible, acceptable, and delivered by trained providers.

Exercise and Integrated Physical-Cognitive Therapies

A 2025 *BMJ* review concluded that exercise and integrated physical-cognitive therapies have favorable benefit–harm balance for chronic low back pain, with benefits likely **moderate** [BMJ](#).

Intervention	Benefit–Harm Balance	GRADE Quality	Recommendation
Exercise therapy	Favorable; likely moderate benefit	Moderate	Offer
Integrated physical-cognitive therapy	Favorable; likely moderate benefit	Moderate	Offer where available
Advice/education	Favorable; likely small benefit	Low–Moderate	Offer as core care
Psychological therapies	Favorable; likely small to moderate benefit	Moderate for CBT/mindfulness; Low for others	Offer selectively

Needling / Acupuncture-Related Interventions

The 2025 *Lancet Rheumatology* review found that **needling** probably reduces disability at long-term follow-up:

Outcome	Effect Estimate	GRADE Quality
Disability	MD −4.8; 95% CI −8.1 to −1.5	Moderate

Source: [PubMed](#)

Interpretation: Needling may remain a conditional option, especially where patient preference, trained providers, safety, and affordability align.

Goal Setting

The 2025 meta-analysis found that goal setting probably reduces disability:

Outcome	Effect Estimate	GRADE Quality
Disability	MD −8.3; 95% CI −12.8 to −3.9	Moderate

Source: [PubMed](#)

Interpretation: Goal setting should be incorporated into rehabilitation plans, self-management, and return-to-function strategies.

Multidisciplinary / Multicomponent Care

The 2025 review found low-certainty evidence that multidisciplinary care may reduce pain intensity, with an available mean difference of –10.1 but incomplete confidence interval data in the supplied summary [PubMed](#).

Intervention	GRADE Quality	Recommendation
Multidisciplinary care	Low	Offer selectively for complex cases
Multicomponent biopsychosocial care	Low–Moderate overall	Continue as WHO-supported care model

NSAIDs

No major post-WHO 2024–2026 effect estimate for NSAIDs was extractable from the supplied evidence. WHO-related reporting indicates NSAIDs may be offered with **moderate-certainty evidence** [MedCentral](#).

Intervention	GRADE Quality	Recommendation
NSAIDs	Moderate	Use selectively, at the lowest effective dose and shortest reasonable duration, considering renal, gastrointestinal, cardiovascular, and drug-interaction risks

Opioids

WHO advises against routine opioid analgesic use for chronic primary low back pain. WHO-related reporting identifies this as one of the few recommendations supported by **moderate-certainty evidence** [MedCentral](#).

The 2026 mindfulness/CBT trial report suggests behavioral interventions may reduce opioid dose in opioid-treated chronic low back pain [Penn State Health News](#).

Intervention	GRADE Quality	Recommendation
Initiating or routinely using opioids	Moderate against	Do not routinely use
CBT/mindfulness for patients already taking opioids	Moderate	Offer as part of opioid-sparing care

Cannabinoids

A 2025 *Nature Medicine* Phase 3 trial of VER-01 full-spectrum cannabis extract reported statistically significant improvements versus placebo in chronic low back pain [Nature Medicine](#).

Outcome	VER-01	Placebo	Result
≥30% pain reduction	54.1%	39.5%	NNTB 6.8; P<0.001
≥50% pain reduction	32.2%	22.8%	P=0.010
≥2-point pain reduction	46.9%	35.6%	P=0.001
Rescue ibuprofen use	10.5 tablets	18.3 tablets	P<0.001

GRADE interpretation: Moderate for short-term efficacy in the studied product and population, but Low for routine clinical adoption because long-term safety, regulatory context, dependence risk, generalizability, and comparative effectiveness remain uncertain.

Recommendation: Do not add routine cannabinoid therapy to WHO-based standard care yet; consider only where legally available, clinically appropriate, and after shared decision-making.

2.4 Evidence Gaps and Future Research Needs

Evidence Gap	Why It Matters	Priority
Limited high-certainty evidence for many non-surgical interventions	Many WHO recommendations are low or very-low certainty	High
Underrepresentation of low- and middle-income countries	WHO guideline is global, but evidence is disproportionately from high-income settings	High
Limited evidence in older adults with multimorbidity	Older adults are at higher risk of adverse effects and disability	High
Limited long-term comparative effectiveness	Patients need durable improvements in function, not only short-term pain relief	High
Limited implementation evidence	WHO recommendations require workforce, financing, referral pathways, and access	High
Limited evidence on digital and telerehabilitation models	Access barriers are substantial; protocols exist but results are pending	Medium–High
Limited opioid tapering and deprescribing trials specific to low back pain	WHO discourages routine opioids, but many patients are already taking them	High
Cannabinoid safety and comparative effectiveness	One positive Phase 3 trial is insufficient for broad routine use	Medium–High
Need for better subgroup evidence	Effects may differ by radicular symptoms, psychological distress, frailty, and comorbidities	High

2.5 Updated Recommendations Based on New Evidence

Original WHO Recommendation	New Evidence Impact	Updated Recommendation as of 29 April 2026	GRADE Quality	Strength
Offer education/advice as part of care	2025 BMJ review supports favorable benefit–harm balance, likely small benefit	Continue to provide education/advice as core care, emphasizing self-management and avoiding overmedicalization	Low–Moderate	Strong as core care
Offer exercise therapy/programmes	2025 BMJ review supports moderate benefit	Continue to offer structured, tailored exercise to improve function and participation	Moderate	Strong
Consider CBT	2025 meta-analysis provides moderate-certainty evidence for	Upgrade confidence: offer CBT when accessible, especially with distress,	Moderate	Conditional

Original WHO Recommendation	New Evidence Impact	Updated Recommendation as of 29 April 2026	GRADE Quality	Strength
	pain and disability reduction	fear avoidance, disability, or opioid use		
Consider mindfulness	2025 meta-analysis and 2026 RCT report support benefit	Offer mindfulness-based interventions as a reasonable option where available and acceptable	Moderate	Conditional
Offer multicomponent biopsychosocial care	New evidence supports integrated physical-cognitive and psychological approaches	Continue to prioritize integrated biopsychosocial care over isolated passive treatments	Moderate	Strong
Consider needling therapies	2025 evidence suggests disability benefit	Continue conditional use where trained providers and safety systems exist	Moderate	Conditional
Consider spinal manipulation	No direct post-WHO chronic-primary-LBP effect estimate extractable; ongoing METRIC trial pending	No change; use only as adjunct within active, person-centred care	Low	Conditional
Offer NSAIDs selectively	No major new extractable post-WHO effect estimate	No change; use cautiously, lowest effective dose, individualized risk assessment	Moderate	Conditional
Avoid routine opioids	2026 CBT/mindfulness report supports opioid-sparing behavioral care	Continue to avoid routine opioids; prioritize CBT/mindfulness and opioid-sparing strategies for patients already on opioids	Moderate	Conditional against routine use
Avoid routine traction, ultrasound, TENS, lumbar braces/supports	No new evidence identified to reverse WHO recommendation	Continue not to routinely offer these passive modalities	Low / Very Low	Conditional against
Avoid routine antidepressants, anticonvulsants, muscle relaxants, glucocorticoids solely for CPLBP	No new evidence identified to reverse WHO recommendation	Continue not to routinely use these medicines solely for chronic primary low back pain	Low / Very Low	Conditional against
Cannabinoids were not supported for routine care in WHO-based summaries	2025 Phase 3 VER-01 trial positive but insufficient for broad recommendation	Do not routinely recommend cannabinoids; consider only in regulated settings with shared	Low–Moderate	Conditional against routine use

Original WHO Recommendation	New Evidence Impact	Updated Recommendation as of 29 April 2026	GRADE Quality	Strength
		decision-making and monitoring		
Telerehabilitation	Protocol evidence only; no results	No recommendation change; may be used pragmatically where it improves access, but comparative efficacy evidence is pending	Very Low	Conditional

2.6 Final Clinical Practice Summary

For adults with chronic primary low back pain, current WHO-based care should be:

1. Person-centred and biopsychosocial

- Address pain, function, mood, sleep, work, participation, beliefs, and social context.

2. Non-surgical and active-first

- Education, reassurance, movement, exercise, self-management, and return to meaningful activity are central.

3. Psychologically informed

- CBT and mindfulness now have stronger post-WHO evidence and should be considered, especially for persistent disability, distress, fear avoidance, or opioid-treated patients.

4. Medication-limited

- NSAIDs may be used selectively.
- Routine opioids, anticonvulsants, antidepressants, muscle relaxants, and glucocorticoids should generally be avoided for chronic primary low back pain alone.

5. Cautious about passive modalities

- Do not routinely use traction, ultrasound, TENS, or lumbar supports.

6. Implementation-focused

- Health systems should invest in primary care, rehabilitation workforce capacity, access to psychological care, referral pathways, and equitable availability of recommended interventions, consistent with WHO's implementation framing [WHO news release](#), [PubMed](#).